Patient Cost-Estimate Automation

A HIPAA-compliant workflow for turning a day's scheduled-patient roster into per-patient cost estimates using ModMed, Availity Essentials, and Doximity Ask.

DISCLAIMER — READ BEFORE PROCEEDING.

1. **Not medical, legal, or financial advice.** This tutorial is shared for educational purposes only and is not a substitute for professional billing, legal, IT, or compliance guidance. AI-generated cost estimates can be wrong — verify every output against your fee schedule, the patient's eligibility data, and your in-house biller or billing consultant before relying on it for real billing.
2. **PHI responsibility stays with your practice.** All responsibility for protecting patient health information (PHI) and complying with HIPAA, payer contracts, plan documents, and applicable regulations remains with your practice. Do not modify your browser, EMR, clearinghouse account, or any other patient-facing system based on this tutorial without first consulting an IT professional and legal counsel who are thoroughly versed in PHI protection and HIPAA compliance for your jurisdiction and your specific configuration.

A step-by-step guide to pulling your day's Eligibility Report out of ModMed, verifying eligibility for every patient in a single batch through Availity (split by payer, pasted into Availity's batch paste box), and generating per-patient cost estimates with Doximity Ask — all within a HIPAA-compliant chain of custody. The reference example uses ModMed and Availity; the architectural pattern adapts to other EMRs and clearinghouses via the companion AI-to-AI Guide.

What You Need

ModMed EMA with **Eligibility Report** accessible from Appt Flow (this is included in standard ModMed configurations — look for the *View Eligibility Report* link with a badge on the right side of the date header). An active Availity Essentials account with Eligibility & Benefits — Batch enabled (included with most Availity subscriptions). A Doximity account with **Doximity Ask** access (free for verified U.S. clinicians). Google Chrome on a Mac or PC. About 20 minutes for the full end-to-end workflow, plus a one-time 5-minute setup step the first time you run it.

HIPAA Note: PHI in this workflow stays inside three vendors that each hold a Business Associate Agreement covering your use — ModMed, Availity, and Doximity. The browser script in Phase 1 runs locally and never transmits anything. The chain is unbroken end to end. **Personal Gmail, generic ChatGPT/Claude/Gemini accounts, and consumer texting apps**

> **are NOT in this chain.** Do not paste Availity output into any of them. Module 10 of the full course covers the email side of this.

> **About the two-prompt design.** The Doximity Ask piece (Phase 3) uses **two prompts**, not one. You run a **Setup Prompt** once to generate your personal **Daily-Use Prompt** with your contracted allowables baked in. You save the Daily-Use Prompt and reuse it for every patient. This separation keeps your contracted rates out of any file you might share publicly and keeps day-to-day use as simple as paste-Daily-Use-Prompt → paste-Availity-row → Ask.

Phase 1: Export the Eligibility Report from ModMed

Step 1: Open the Eligibility Report

Log in to ModMed EMA. Click **Appt Flow** in the top navigation. On the right side of the date header (where it says e.g. *Friday, May 29, 2026*), click **View Eligibility Report**. The page loads with all scheduled patients for the active filter, with insurance information already attached to each.

Step 2: Set the Filter and Pagination

Set the filter to the date range you want verified — most practices run this once a week for the upcoming 7-day window. Set pagination to show **ALL rows** (highest available value in the per-page dropdown). The script reads only what is visible — anything hidden behind pagination is skipped.

Step 3: Open the Chrome Developer Console

Press **Cmd+Option+J** (Mac) or **Ctrl+Shift+J** (Windows). Chrome will show a warning about pasting code into the console. Type `allow pasting` and press Enter.

The first time you run this script on a multi-payer day, Chrome will also prompt: *"This site is attempting to download multiple files."* Click **Allow**. Chrome remembers this for the rest of the session.

Step 4: Run the Extraction Script

Open the file **1_Extract_Eligibility_Report.js** from this bundle. Select all the text (Cmd+A / Ctrl+A), copy it, paste it into the Chrome console, and press Enter.

The script walks the Eligibility Report table, groups patients by payer, and writes **one plain-text file per unique payer** to your Downloads folder. Each file contains only the two fields Availity's batch paste box expects — patient insurance ID and date of birth — with no header row:

```
W123456789, 01/15/1962
A987654321, 07/22/1948
B445566778, 03/03/1971
```

Filenames are payer-derived, e.g.:

- `availity_medicare_of_texas.txt`
- `availity_bcbs_ppo.txt`
- `availity_aetna_choice_pos.txt`

Watch the console output. You should see an extraction summary listing each payer and patient count per payer, followed by one *Downloaded: ...* line per file.

Phase 2: Run the Batches Through Availity

Step 5: Log in to Availity Essentials

Open `availity.com` and sign in. Navigate to **Patient Registration → Eligibility and Benefits → Batch** (the exact menu path may vary; you're looking for the option to paste a list of patients rather than entering one patient at a time).

Step 6: Paste Each Payer's File Separately, One at a Time

For each payer, you'll submit one batch:

1. In Availity, select the payer for this batch.
2. Open the matching `availity_<payer>.txt` file from your Downloads folder.
3. Select all (Cmd+A / Ctrl+A), copy (Cmd+C / Ctrl+C), and paste (Cmd+V / Ctrl+V) into Availity's batch eligibility paste box. Because the file has no header row, your select-all grabs only data — nothing to delete before submitting.
4. Submit the batch.
5. Move on to the next payer's file.

One payer per batch — Availity's batch endpoint runs against one payer's roster per submission, so mixing payers in one paste will either be rejected or processed incorrectly. Splitting by payer up front (which the Phase 1 script does for you) matches Availity's expected workflow.

Availity will validate each batch and flag any malformed rows — missing member IDs, badly formatted dates of birth. Fix those rows directly in the .txt file (or in ModMed) and re-paste only the failed subset.

Step 7: Submit and Wait

Submit each validated batch. Availity processes most batches in about three minutes; a 100-patient batch typically returns inside five. Per-payer response time varies, and an occasional payer is slow enough that some rows return as **Pending**. Pending rows resolve within a few hours — you can re-export the batch result later and re-process the subset that was pending.

Step 8: Download the Response

When each batch completes, download the response CSV. **One response CSV comes back per batch you submitted — so one per payer.** The response will include, for each patient: member status, plan name and type, copay tier, deductible amount and remaining balance, out-of-pocket maximum and remaining, in-network coinsurance percentage, and any prior-authorization flags on the plan. These response CSVs are the input for Phase 3.

Three de-identified per-payer samples are included in this bundle to show you the data shape and exercise different plan-type scenarios before you run real data:

- **Sample_Availity_Output_Medicare.csv** — traditional Medicare batch. Shows Active, Pending, Inactive, deductible variations, and Plan G / Plan F Medigap edge cases.
- **Sample_Availity_Output_BCBS_PPO.csv** — BCBS commercial PPO batch. Shows varied deductibles, an auth-required edge case for outpatient surgery, an OOP-nearly-met scenario, Pending, and Inactive.
- **Sample_Availity_Output_Humana_MA_HMO.csv** — Humana Medicare Advantage HMO batch. Shows Auth Info Unknown, PCP referral required, $0-copay scenarios, Pending, and Inactive.

Open them now to see the data shape. Each file represents one batch's response — single-payer, matching the one-paste-per-payer pattern from Phase 1.

Phase 3: Generate Per-Patient Cost Estimates with Doximity Ask

Step 9 (one-time): Generate Your Personal Daily-Use Prompt

Open **DoximityAsk_Setup_Prompt.md** (or the matching **.pdf**) from this bundle. Sign in to `doximity.com` and open Doximity Ask. Click **+ New Question**. Copy the **Setup Prompt block** from the file (the text between the `=== START OF SETUP PROMPT ===` and `=== END OF SETUP PROMPT ===` markers) and paste it into the Doximity Ask chat box.

Below the Setup Prompt, paste:

- **Your raw fee schedule** — a table or list with CPT codes, descriptions, and your contracted allowables for each payer you bill. Any format is fine: markdown table, CSV-style text, or attach a PDF via the paperclip icon.
- **Your practice details** — practice name, solo/group, specialty, city, state. Optional. If you skip these, the Daily-Use Prompt comes back with placeholders for you to fill in manually.

Hit **Ask**. Doximity Ask returns a complete **Daily-Use Prompt** with your fee schedule and practice details baked in. **Copy and save it.** Use Doximity Ask's *Saved Prompts* feature, a password manager, or any private encrypted note. This is the prompt you reuse for every patient — you do not run the Setup Prompt again.

> **IMPORTANT.** Your saved Daily-Use Prompt contains your real contracted allowables. Those rates are confidential under most insurance contracts. Do not commit the Daily-Use Prompt to a public repo, post it anywhere, paste it into ChatGPT / Claude / Gemini / Grok personal accounts, or email it through personal Gmail. Keep it inside the Doximity BAA chain or a private encrypted store.

Step 10 (per patient): Paste Daily-Use Prompt + Patient Row

In a **fresh Doximity Ask session** (click **+ New Question** — don't reuse the setup session), paste your saved Daily-Use Prompt. Then, immediately after it, paste the patient's row from your Availity output CSV — including the header row, so Doximity Ask knows which column is which. Alternatively, click the paperclip icon and attach the full Availity output PDF and ask Doximity Ask to process a specific patient by name.

Hit **Ask**.

Step 11: Review the Output

Doximity Ask returns:

- A one-line patient summary (name, plan, member status, deductible remaining, OOP max remaining)
- A per-procedure breakdown — every CPT code in your fee schedule with its allowable, what applies to deductible, coinsurance, copay assigned, and patient owed
- The total to collect up front today, with line-by-line auditable arithmetic. For mutually exclusive code groups, both combinations are computed and the conservative (higher) one is recommended for collection.
- Flags (auth required, inactive coverage, missing or ambiguous data, PCP referral required, OOP max reported as met) requiring staff review
- A one-line front-desk-ready summary you can paste directly into the appointment note or read aloud to the patient at check-in

Step 12: Repeat or Batch

For a single patient, that is the workflow. For a whole batch, you have two options:

- **Run patients one at a time** if you want to review each estimate before it goes into the chart. About 30 seconds per patient. Open a fresh Doximity Ask session for each — don't reuse the previous chat, because prior conversation context can interfere with the next patient's calculation.
- **Attach the full Availity output and ask Doximity Ask to process every row** if you trust the prompt's guardrails for your specialty. About 2-3 minutes for a 40-patient batch. Spot-check 3-5 rows for sanity before using the rest.

Troubleshooting

"Cannot find #reportTable" — You are not on the Eligibility Report page, or the table has not finished loading. Navigate to Appt Flow → View Eligibility Report and wait for the table to render before running the script.

"Patient column not found" or "Policy Number column not found" — The table loaded, but its column headers do not match what the script expects ("Patient", "Payer", "Policy Number"). If ModMed has renamed columns in your configuration, adjust the header-matching lines near the top of the script.

Script returns 0 rows — Pagination is likely still set to a low number, or your filter is too narrow. Set pagination to "show all" and confirm the rows you want are visible, then re-run. If many rows show as "skipped" in the console output, the script couldn't find a member ID and/or DOB on those rows — check ModMed has both fields populated.

Chrome blocks the downloads — On the first multi-payer run, Chrome prompts to allow multiple downloads. If you missed the prompt, click the small icon in the address bar (a download or shield icon) and grant permission, then re-run the script.

Availity rejects the paste — Check the validation error. Usually a malformed DOB (Availity wants MM/DD/YYYY) or a missing member ID. Fix the offending lines in the .txt file and re-paste only the failed subset.

Some Availity rows come back as Pending — Normal. The payer has not responded yet. Re-run the batch in a few hours and just process the rows that were pending.

The Setup Prompt asks you for an Availity report — Doximity Ask conflated the two prompts. Start a fresh session, paste the Setup Prompt cleanly with your fee schedule below it (but no Availity data), and try again. Also confirm you pasted the Setup Prompt and not the Daily-Use Prompt.

The returned Daily-Use Prompt has columns for payers you did not provide — Doximity Ask hallucinated those columns. Delete them manually from the Daily-Use Prompt before saving. The Setup Prompt instructs Doximity Ask not to do this, but check anyway. Patients on those payers will route to the Medicare fallback, which is the correct behavior.

Doximity Ask output is missing fields — The Availity row you pasted is missing those fields. Pending rows have null deductible / coinsurance / OOP data. Doximity Ask correctly refuses to fabricate a number and flags the patient for re-check. Do not "fill in" the missing data manually.

Doximity Ask estimate looks wrong — First, verify your fee schedule in the saved Daily-Use Prompt matches your actual contracts. Second, verify the mutex pairs listed are right for your specialty. Third, run the same patient again in a fresh session; the calculation should be deterministic. If it still looks wrong, have your biller review — the Daily-Use Prompt includes line-by-line math for exactly this audit purpose.

What This Replaces

Before automation, the same daily roster looks like this: a front-desk person opens Availity, looks up each patient one at a time, copies the deductible and copay information into a spreadsheet, then opens a calculator and works out an estimate using a separate fee schedule. About six to eight minutes per patient. For 40 patients a week, most of a workday.

With the automation: about 15 seconds of script time in Phase 1, three to five minutes of Availity processing per payer in Phase 2 (parallelizable — you can submit all payer batches at once), and 30 seconds per patient in Phase 3 (or a single batch run if you trust the guardrails). For 40 patients, the total clock time is under 30 minutes — most of which is Availity processing while you do something else.

The compliance posture is also stronger than the manual workflow. Manual lookup means a staff member is reading patient eligibility information on screen and re-typing it into a spreadsheet — a step where transcription errors quietly enter the chart. The automated workflow moves structured data between vendors that already have BAAs with you, with no manual re-typing in the middle.

Companion document: DoximityAsk_AI_to_AI_Guide.pdf. Course module: Module 5 — Insurance Verification & Patient Cost Estimates. — Bundle v6.